

3/2/2025 via FDMS

Seth Hyman

Home sleep testing and CPAP devices including BPAP and ASV should be available to patients with much less of a burden than current treatment protocols and insurance companies require. There are very few contraindications for these devices overall and AI is nearly perfect at making the determination. Get with the program and streamline the procurement process with things like this. As a credentialed Polysomnographer I should be able to own and operate CPAP specific distribution clinics and websites without having to grease the palms of an MD to keep the doors open. The oversimplification here is intended to get the discussion started. There are a lot of technicians in sleep diagnostics who could effectively saturate the population with a much higher percentage of high quality sleep-hours if deregulation were to take place. It'd be comparable to the impact that legalizing marijuana had on the cartel shipping weed into the US, but this outcome would be realized on the Medicare fraud prevention side as well as the amount of resources needed to review and approve insurance claims for PAP devices, oxygen, sleep testing, and other devices. You want to save Medicare and Social Security, so here it is. With respect to any medical practice that treats 95% of its patients with the same form of treatment, there's no reason to bill insurance for MD assessments and expertise. I'm alluding to the fact that 9.5 out of 10 patients who present to the sleep clinic are initially treated with CPAP. The paperwork generated in this process is asinine.